

AdvocateAuroraHealth

Title: AAH System Incident Notification and Incident Command Activation		Document Number: 2522
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Content Applies to Patient Care: (Select all that apply) ☐ Adults ☐ Pediatrics (Under 18)	Content Applies to: (Select One) ☐ Clinical ☒ Administrative	Next Review Date: 09/12/2022
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Scope: ☒ AAH System ☐ AAH IL Only ☐ AAH WI Only ☐ Site Only (Location Name): ☐ Department Only (Department Name):		

I. PURPOSE

The purpose of this policy is to provide guidance to all sites and system entities when unplanned or unusual incidents occur necessitating notification to system Emergency Management.

The policy also lends guidance for sites and system entities to promote communication to system Emergency Management regarding planned events or exercises to hardwire communication processes.

II. SCOPE

This policy applies to Advocate Aurora Health, Inc. and any entity or facility owned or controlled by Advocate Aurora Health.

III. DEFINITIONS/ABBREVIATIONS

ASCC – Advocate Aurora Security Command Center
CMO – Chief Medical Officer
HICS – Hospital Incident Command System
ICS – Incident Command System
IMT – Incident Management Team
PCC – Patient Command Center
RHCC- Regional Hospital Coordination Center

IV. POLICY

Notification by a site or system entity may necessitate the activation of the System Incident Command System (System ICS) to render support to a site/entity.

Advocate Aurora Health will utilize the System (Hospital) Incident Command System (ICS) to provide site support or manage an incident at the corporate level. The ICS enables effective and efficient incident management via the

integration of a combination of facilities, equipment, personnel, service lines, procedures, and communications operating within a common organizational structure. The goal is to successfully support the affected entity to manage the incident, minimize impact, and return the entity and administrative support centers to normal operational status.

This policy may be used in conjunction with the Advocate Aurora Business Continuity Plan or Health Informatics and Technology Disaster Recovery Plan or Cybersecurity Incident Response Plan.

V. PROCEDURE

A. Notification of System Emergency Preparedness:

1. When a site is impacted by an unplanned or unusual event (or in the event of a planned event/exercise), the site team shall:
 - a) Address immediate needs to provide safety for patients, visitors, and staff.
 - b) Convene a response to address the incident (e.g. HICS, IMT).
 - c) Call the ASCC (847-723-2722) to report the incident and provide the following information if possible:
 - (1) Type of incident/occurrence
 - (2) When incident/occurrence began
 - (3) Impact to facility
 - (4) Name and phone number of the caller (or designee)
 - d) The ASCC lead dispatcher will notify the system Emergency Preparedness Director or designee and ASCC leadership.
 - e) The system Emergency Preparedness director or designee will contact the caller if additional information as needed.
 - f) For an unplanned or unusual event, the system Emergency Preparedness Director or designee will notify:
 - (1) System Chief Medical Officer (CMO) or designee
 - (2) System Business Continuity Assessment Team
 - g) The System CMO or designee will determine the need to activate the System Incident Command System – either in person or virtually.

- h) The Emergency Preparedness Director will notify the System Leadership Team of Incident Command activation (in person or virtual) via a message notification system.
- i) In the event of a site declaring Bypass to the Advocate Patient Command Center (PCC), the PCC will notify the ASCC with the information received. The ASCC will notify the system Emergency Preparedness Director or designee and ASCC leadership.

B. Activation of System Incident Command:

1. The System Incident Management Team (IMT) will be convened in person or virtually. A System Incident Commander will be determined based on the nature of the emergency and the availability of staff. The System Incident Commander must be present at the System Incident Command Center or on the System Incident Command Conference Call. The System Incident Commander will determine the composition of the System Incident Command Team that will be assembled based on the scope of the disaster/emergency circumstance.
2. The System Chief Medical Officer or designee may call upon individuals or designees representing the following positions to convene the "System Incident Command Team". The System team members notified will be *incident dependent*.
 - a) Chief Medical Officer (CMO)
 - b) Chief Operating Officer (COO)
 - c) Chief Financial Officer (CFO)
 - d) Chief Information Officer (CIO)
 - e) Chief Compliance Officer (CCO)
 - f) Chief Human Resource Officer
 - g) Chief Marketing Officer (PAM)
 - h) Chief External Affairs Officer
 - i) Chief Nursing Officer
 - j) Chief Legal Officer
 - k) Chief Spiritual Officer
 - l) Senior Vice President Human Resources
 - m) Senior VP of Clinical Support Operations/Clinical Transformation
 - n) Senior Vice President, Support Services

- o) Vice President, Risk Management
 - p) Vice President, Security Services
 - q) Vice President, Facilities Services
 - r) Vice President, Supply Chain
 - s) Vice President, Nursing Operations eICU / Patient Command Center
3. The location(s) of the System Incident Command Center will be identified by the System Incident Commander depending on the circumstances. Options include but are not limited to:
- a) Virtual Command (630-556-1696 ,,, 659600355#)
 - b) Advocate Support Center- Downers Grove Conference Room 713
 - c) Aurora Administrative Office – Milwaukee
 - d) Advocate Security Command Center (neighboring telepresence room) in Park Ridge (LGH) 5th Floor – Room E 592

Each System Incident Command location is equipped with a System Incident Command Kit. The kit contains pre-packaged Incident Command position vests with clipboards containing Job Description Checklists, Incident Command Documentation Forms and system policies. The location of the kits are as follows:

- Advocate Support Center- Downers Grove Conference Room 713 – 7 North Closet (copy machine area)
 - Aurora Administrative Office – Milwaukee – in the 24/7 Call Center
 - Advocate Security Command Center (neighboring telepresence room) in Park Ridge (LGH) 5th Floor – Room E 592 – Computer Work Room
4. The System Incident Command Team will support the sites/entity by assessing emergent circumstances, develop and communicate a system-wide plan of action as needed, and re-assess ongoing situation status.
5. The Site/Entity Incident Commander or designee will maintain communications with the System Incident Management Team.
6. Communications will be provided to appropriate leadership utilizing the Hospital/Healthcare Incident Command System (HICS)

documentation forms. The frequency of communication will be determined by the System Incident Command Team according for the nature of the incident.

C. Community Integration

If the System Incident Commander determines that an effective response to the incident requires community assistance, the applicable community resources will be contacted to make additional resources available to the system.

This may be accomplished directly to local municipal agencies or State/City agencies or via a system hospital site or Regional Hospital Coordination Center (RHCC). Hospital sites and RHCCs have direct access to local and state response agencies.

RHCCs affiliated with Advocate Aurora Health are; Christ Medical Center, Illinois Masonic Medical Center and Sherman Hospital.

D. Termination Procedure

When a site/entity determines that the incident has concluded, the Site Incident Commander will notify the System Incident Commander.

When the System Incident Commander determines that the incident has concluded, the System Incident Commander will announce the termination of the System Incident Command Team. The termination will be communicated to appropriate leadership via a message notification system.

A debriefing will be conducted by the System Incident Command team to identify opportunities for future improvement after the completion of each System Incident Command activation. An After-Action Report will be completed, reviewed by the Emergency Preparedness Council and revisions made as directed. The Incident and After-Action Plan will be reported through the Advocate Aurora Support Center QMOC.

As a part of the ongoing re-assessment process, the involved site/entity will submit a copy of their respective After-Action Report via their Emergency Management/Preparedness liaison to the System Emergency Preparedness Director.

VI. CROSS REFERENCES

Business Continuity Plan
Technology Disaster Recovery Plan
Cybersecurity Incident Response

VII. RESOURCES AND REFERENCES

Not Applicable

VIII. ATTACHMENTS

Not Applicable